4.5

PAEDIATRICS

Ear, Nose, and Throat Infections

CHAPTER 4.5	EAR, NOSE, AND THROAT INFECTIONS		
Infections and Likely Organisms	**Suggested Treatment**		**Notes**
	Recommended	**Alternative**	
Tonsillopharyngitis			
Bacteria: *S pyogenes* (GABHS) Other Streptococci (Groups C, G) *Mycoplasma pneumoniae* *Chlamydia pneumoniae* **Viruses:** *Epstein-Barr virus* *Coxsackie viruses*	Phenoxymethylpenicillin 10 - 15 mg/kg/dose PO q6H for 10 days **OR** Amoxicillin 15 mg/kg/dose PO q8H for 5 - 7 days	**Penicillin Allergy:** Erythromycin Ethyl Succinate (EES) 20 mg/kg PO q12H	• Follow-up for complications like acute rheumatic fever, carditis and acute post-streptococcal glomerulonephritis.
Acute Rhinosinusitis			
Virus: Rhinoviruses (Common cold)			• Vast majority are caused by rhinoviruses and settle by 2 - 3 weeks. The top 3 bacteria account for > 90% of cases of secondary bacterial infection.

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Bacteria: *Streptococcus pneumoniae* *Haemophilus influenzae* *Moraxella catarrhalis* *S pyogenes* *S aureus* Anaerobes	Amoxicillin 25-45 mg/kg/dose PO q12H **OR** Co-amoxiclav 20-25 mg/kg (Amoxicillin component) PO q12H	**Penicillin Allergy:** Clindamycin 10 mg/kg/dose (max 300 mg) PO q8H	• Prolonged therapy up to 10-14 days may be required.
	Severe: Ampicillin/Sulbactam 50 mg/kg/dose (Ampicillin component) IV q6H	Ceftriaxone 50 mg/kg/dose IV q24H	
Otitis Media			
Acute otitis media *Streptococcus pneumoniae* *Haemophilus influenzae* *Moraxella catarrhalis* *S pyogenes (GABHS)* *Staph aureus* *Pseudomonas aeruginosa*	**First line:** Amoxicillin 40-45 mg/kg/dose PO q12H **Second line:** Co-amoxiclav 20-25 mg/kg/dose (Amoxicillin	**Penicillin Allergy:** Azithromycin 10 mg/kg/dose PO q24H (max 500 mg) for 1 day; then 5 mg/kg/dose (max 250 mg) q24H for 4 days **OR**	• Duration of oral therapy ranges from 5 days (> 2 years old) to 10 days (< 2 years old). • Shorter duration for Azithromycin.

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	component) PO q12H (max 875 mg PO q12H) **OR** Cefuroxime 15 mg/kg/ dose PO q12H	Erythromycin Ethyl Succinate (EES) 20 mg/kg/dose PO q12H	
Chronic suppurative otitis media *Pseudomonas aeruginosa* *Proteus mirabilis* *Klebsiella pneumoniae* *Escherichia coli* *Staphylococcus aureus* *Streptococcus pneumonia* *Anaerobes – Bacteroides, Propionibacterium spp.*	Ear toileting **PLUS** Topical quinolone e.g., Ciprofloxacin 0.3%		• No consensus on optimal treatment for CSOM in children, including topical or systemic antibiotic use. • Topical antibiotics and ear toileting are the preferred treatments now.
Mastoiditis			
Bacteria: *Streptococcus pneumoniae* *Haemophilus influenzae* *Moraxella catarrhalis* *S pyogenes* (GABHS)	Co-amoxiclav 25 mg/kg/ dose (Amoxicillin component) IV q8H	Ceftriaxone 50 mg/kg/ dose IV q24H + Metronidazole 7.5 mg/kg/dose IV q8H	

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Staph aureus (MSSA) Anaerobes			